

Patient Intake and Triage Standard Operating Procedure

Meridian Health Partners

Standard Operating Procedure: Patient Intake and Urgency Triage

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Effective Date:	2024-07-01	Owner:	Operations Policy Lead

1.0 Purpose

1.1 This Standard Operating Procedure (SOP) establishes the mandatory data capture requirements, patient urgency banding scheme, and associated response protocols for all initial patient contacts at Meridian Health Partners (MHP) sites.

1.2 The objective is to ensure consistent, accurate, and timely patient intake, appropriate clinical triage, and proper documentation across all 14 MHP outpatient clinics.

2.0 Scope

2.1 This SOP applies to all MHP personnel involved in the initial contact and intake of prospective or returning patients, including but not limited to Front Desk Coordinators, Call Center Agents, Patient Navigators, and Triage Nurses.

2.2 This procedure covers initial contact via telephone, patient portal messages, walk-ins, and digital intake forms.

3.0 Definitions

3.1 First Contact: The initial interaction where a prospective or returning patient communicates a need or request for service to Meridian Health Partners personnel.

3.2 Business Hours: Monday through Friday, 08:00 to 17:00 local time, excluding observed federal holidays.

3.3 MeridianCare Connect: MHP's designated Electronic Health Record (EHR) system.

4.0 Procedure

4.1 Mandatory Fields on First Contact

Upon first contact, the following fields must be captured in MeridianCare Connect by the responsible agent:

1. Patient Full Legal Name: As per government-issued identification where possible.
2. Date of Birth: DD/MM/YYYY format.
3. Primary Contact Telephone Number: Must include area code.
4. Brief Reason for Contact (Chief Complaint): A concise summary of the patient's stated need or symptom (e.g., "annual physical," "right knee pain," "medication refill request").
5. Preferred Language: Patient's stated preference for communication.
6. Sex Assigned at Birth / Gender Identity: As identified by the patient.

4.2 Urgency Banding Scheme and Response Times

All patient contacts must be assigned an urgency band (1-4) based on the stated chief complaint and trigger phrases. The responsible agent must document the assigned band and the rationale (e.g., trigger phrases used) in MeridianCare Connect.

4.2.1 Band 1: Routine

1. Definition: Non-urgent medical concerns, general inquiries, or administrative requests that do not require immediate clinical intervention.

2. Trigger Phrases (Examples):

- * "I need to schedule my annual physical."
- * "I want to discuss my cholesterol levels."
- * "I need a refill for my blood pressure medication, I have 10 days left."
- * "I'd like to schedule a follow-up for my stable diabetes."
- * "General check-up."

3. Target First-Response Time: Contact by scheduling staff within 24 business hours.

4. Service Provision Goal: Appointment or resolution scheduled within 14 calendar days.

4.2.2 Band 2: Priority

1. Definition: New or worsening symptoms causing significant concern but not immediately life-threatening, or time-sensitive administrative requests.

2. Trigger Phrases (Examples):

* "I have a new rash that's bothering me."

* "I received an abnormal lab result and want to discuss it." (Non-critical result per MHP Lab Result Review Policy MHP-POL-LAB-003)

* "My medication refill request is for a critical medication (e.g., insulin, blood thinner) and I have less than 3 days supply remaining."

* "I'm experiencing discomfort after my surgery last week, but it's not severe."

* "My child has a fever of 101°F for two days, but is otherwise active."

3. Target First-Response Time: Contact by scheduling or clinical staff (Registered Nurse or licensed clinician) within 4 business hours.

4. Service Provision Goal: Appointment or resolution within 3 business days.

4.2.3 Band 3: Urgent

1. Definition: Acute medical conditions requiring prompt clinical evaluation to prevent significant deterioration, but not necessitating emergency department care.

2. Trigger Phrases (Examples):

* "I have severe pain, like 7/10 or more, in my abdomen."

* "I suddenly can't move my arm properly."

* "I have a deep cut that might need stitches."

* "I'm experiencing severe vomiting and diarrhea and feel very dehydrated."

* "My chronic heart condition symptoms are suddenly much worse."

* "I suspect I may have a fracture."

3. Target First-Response Time: Clinical staff (Registered Nurse or licensed clinician) contact within 15 minutes.

4. Service Provision Goal: Appointment or appropriate referral within 24 hours.

4.2.4 Band 4: Emergent

1. Definition: Life-threatening medical emergencies or conditions requiring immediate emergency medical attention. MHP outpatient clinics are not equipped to handle such cases.

2. Trigger Phrases (Examples):

* "I cannot breathe."

* "I have severe chest pain radiating to my arm."

* "I'm experiencing uncontrolled bleeding."

* "Someone just lost consciousness."

* "Sudden severe weakness or numbness on one side of the body."

* "Sudden, excruciating headache."

* "I am thinking of harming myself or others."

* "Major trauma (e.g., severe car accident, fall from height)."

3. Target First-Response Time: 0 minutes. Immediate transfer of call to 911 (or local emergency services) or direct instruction to the caller to proceed to the nearest Emergency Department.

4. Service Provision Goal: Immediate transfer to emergency services. MHP personnel must remain on the line until transfer or confirmation of patient action.

4.3 Handling a Patient Who Declines to Give an Insurance ID

4.3.1 If a patient declines to provide an insurance ID, the responsible agent must:

1. Inform the patient that providing insurance information allows MHP to verify benefits and bill their insurer directly, potentially reducing their out-of-pocket costs.
2. If the patient still declines, explain that Meridian Health Partners operates on a self-pay model for services not covered by insurance or where insurance information is not provided.
3. Inform the patient of the estimated self-pay rate for the requested service (e.g., \$150.00 for a New Patient Office Visit, Level 3).
4. Offer information regarding the Meridian Health Partners Financial Assistance Program (FAP) and direct them to the MHP Billing Department or Patient Financial Counselor, Dr. Elias Vance, for details.
5. Proceed with scheduling the appointment, but flag the patient's record in MeridianCare Connect with a "SELF-PAY PENDING" alert.
6. Require the patient to sign a "Self-Pay Responsibility Acknowledgment" form prior to the service delivery. This form explicitly states the patient's financial responsibility.
7. Collect upfront payment for the anticipated service based on standard self-pay rates (e.g., office visit fee) at the time of check-in, unless the patient has a pre-approved FAP application.

4.3.2 Exception for Urgent and Emergent Cases (Bands 3 & 4):

1. No patient presenting with an Urgent (Band 3) or Emergent (Band 4) condition will be denied or delayed care solely due to lack of insurance information or inability to provide upfront payment.
2. For Band 3 cases, intake and triage proceed immediately. Financial counseling and self-pay procedures are initiated concurrently or post-stabilization of the patient.
3. For Band 4 cases, immediate referral to emergency services takes precedence. Financial discussions are deferred until appropriate.

4.4 Record-Keeping Requirement

- 4.4.1 All interactions and data points captured during first contact must be meticulously documented in MeridianCare Connect.
- 4.4.2 The system will automatically apply a date and time stamp to each entry.
- 4.4.3 The unique Agent ID of the MHP personnel making the entry must be associated with the record.
- 4.4.4 The assigned urgency band and the specific trigger phrases or rationale used for the banding decision must be recorded in the "Chief Complaint" or "Triage Notes" section.
- 4.4.5 If insurance information was declined, a specific note detailing the patient's refusal, the discussion regarding self-pay rates, and whether FAP information was provided must be entered.
- 4.4.6 All communication attempts (e.g., phone calls, messages) must be logged, including the outcome of the attempt and any next steps required.
- 4.4.7 All patient records and associated documentation will be retained in accordance with the Meridian Health Partners Data Retention Policy (Policy MHD-DTR-001).

5.0 Exceptions

5.1 Any deviation from this SOP requires prior approval from the Operations Manager, Ms. Clara Benton, or the Chief Medical Officer, Dr. David Chen, and must be documented in MeridianCare Connect with a clear justification.

6.0 Responsibilities

6.1 Operations Policy Lead: Responsible for the creation, review, and revision of this SOP.

6.2 Operations Manager: Responsible for ensuring staff compliance and providing necessary resources.

6.3 Site Managers: Responsible for training and monitoring staff adherence to this SOP at their respective sites.

6.4 All MHP Personnel: Responsible for understanding and strictly adhering to this SOP.

7.0 References

7.1 MHP-POL-LAB-003: MHP Lab Result Review Policy

7.2 MHD-DTR-001: Meridian Health Partners Data Retention Policy

7.3 MHP-SOP-FIN-002: Financial Assistance Program (FAP) Application Process

8.0 Revision History

Version	Date	Author	Description of Change
1.0	2023-01-15	Elias Vance	Initial release.
2.0	2024-03-01	Clara Benton	Revised trigger phrases for clarity, added specific response times per band, introduced mandatory fields section, detailed insurance decline procedure, and updated record-keeping requirements.
2.1	2024-07-01	David Chen	Minor grammatical corrections. Clarified "Target First-Response Time" and "Service Provision Goal" for each band. Added specific mention of MHP outpatient clinic limitation for emergent cases. Specified personnel for approval of exceptions. Removed "may vary."